CHIEF COMPLAINT: Episode of painless gross hematuria with passage of vermiform blood clots.

HISTORY: A 57-year-old male smoker (30 pack-years) presents for urgent urological evaluation following two episodes of gross, bright red hematuria three days ago. He reports mild, dull right flank ache but denies fevers, chills, dysuria, frequency, or urinary hesitancy. He has lost 4 kg of unintentional weight over the past two months.

PHYSICAL EXAMINATION:

Vital Signs: BP 142/88 mmHg, HR 74 bpm, afebrile.

Abdomen: Soft, non-distended. On bimanual palpation of the right upper quadrant, a firm, non-tender, ballotable right flank mass is appreciated. Left flank is non-tender.

Genitourinary: Normal circumcised phallus without urethral discharge; bilateral testes descended and normal without palpable masses or varicocele. Digital rectal exam reveals a smooth, non-tender, approximately 25-gram prostate without nodules.

IMAGING WORKUP:

Triple-Phase Computed Tomography Urogram:

1. Kidneys: Demonstrates a 5.8 x 6.2 x 5.4 cm solid, exophytic, heterogeneously enhancing mass arising from the lower pole of the right kidney. The mass demonstrates central necrosis, characteristic hypervascularity in the corticomedullary phase (attenuation rising from 32 HU pre-contrast to 138 HU post-contrast), with washout in the excretory phase, highly suggestive of clear cell renal cell carcinoma (RCC).

2. Vascular: The right renal vein and inferior vena cava are completely patent without tumor thrombus.

3. Left Kidney: Completely normal with normal excretion.

4. Collecting System: No filling defects in the renal pelvis, ureters, or bladder.

IMPRESSION AND PLAN:

1. Right renal mass, clinical stage T1bN0M0, suspicious for renal cell carcinoma.

2. In-office flexible cystoscopy performed today: Normal anterior and posterior urethra, normal bladder mucosa with no intravesical tumors; clear efflux observed from both ureteral orifices.

3. Order chest CT to complete oncological metastatic staging.

4. Schedule for robotic-assisted laparoscopic right partial vs radical nephrectomy.